

1. Administrative & Study Identification

Study Title: An Investigational Immuno-therapy Study of Nivolumab, Compared to Placebo, in Patients With Bladder or Upper Urinary Tract Cancer, Following Surgery to Remove the Cancer
Official Title: A Phase 3 Randomized, Double-blind, Multi-center Study of Adjuvant Nivolumab Versus Placebo in Subjects With High Risk Invasive Urothelial Carcinoma (CheckMate 274: CHECKpoint Pathway and nivoluMab Clinical Trial Evaluation 274) **Short Title/ Acronym:** CheckMate-274 (CA209-274) **Protocol Number:** CA209-274 **NCT Number:** NCT02632409 **Posting ID:** 265719595791392171 **Sponsor / Company Name:** Bristol Myers Squibb **Investigational Product (Drug Preferred Name/UMLS Preferred Name):** Nivolumab **Phase of Development:** Phase III (phase_id: PHASE3) **Medical Monitor:** Clinical Operations Lead, Bristol Myers Squibb

2. Study Synopsis & Rationale

2.1 Study Objective Summary/Purpose: The purpose of this study is to determine the effectiveness and safety of Nivolumab compared to placebo in participants who have undergone radical surgery for invasive urothelial cancer. **Primary Objective:** To evaluate the efficacy of adjuvant nivolumab compared with placebo in preventing or delaying disease recurrence (Disease-Free Survival [DFS]) in the intent-to-treat (ITT) population and in patients with tumors expressing PD-L1 $\geq 1\%$. **Secondary Objectives:** To evaluate Overall Survival (OS), Non-urothelial tract recurrence-free survival (NUTRFS), and Non-urothelial tract recurrence-free survival in PD-L1 Expression $\geq 1\%$ Population.

2.2 Background & Rationale To address the unmet medical need in patients with high-risk invasive urothelial carcinoma (UC) who have undergone radical resection but remain at high risk of recurrence. Up to 60% of patients who undergo radical cystectomy are not candidates for adjuvant cisplatin therapy or have already received it prior to surgery. This study tests the hypothesis that prophylactic immunotherapy with the PD-1 inhibitor nivolumab can help the immune system recognize and attack residual cancer cells, significantly reducing the risk of recurrence.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1 (Stratified by tumor PD-L1 expression, lymph node status, and

prior neoadjuvant cisplatin chemotherapy) **Criteria Type:**
PHASE_REQUIREMENT (Criteria Text: Trial must be in PHASE3)

3.2 Planned Enrollment & Duration Trial Status:

ACTIVE_NOT_RECRUITING (Active but not currently recruiting)

Targeted Enrollment: 709 randomized patients **Number of Sites:**

Global and Regional multicenter breakdown **Estimated Study Start:**

2016 **Estimated Primary Completion:** 2020 (Extended survival follow-ups ongoing into 2025/2026)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Must have had invasive urothelial cancer at high risk of recurrence originating in the bladder, ureter, or renal pelvis. 2. Must have had radical surgical resection (e.g. radical cystectomy), performed within the last 120 days. 3. Must have disease-free status as determined by imaging within 4 weeks of dosing. 4. Patients who have not received prior neoadjuvant cisplatin chemotherapy must be ineligible for or refuse cisplatin-based adjuvant chemotherapy. 5. Tumor tissue must be provided for biomarker analysis.

4.2 Exclusion Criteria 1. Partial bladder or partial kidney removal (e.g., partial cystectomy or partial nephrectomy). 2. Secondary treatment (e.g., adjuvant systemic chemotherapy for bladder cancer) following surgical removal of the cancer/bladder cancer. 3. Subjects with active, known, or suspected autoimmune disease. 4. Prior malignancy active within the previous 3 years except for locally curable cancers that have been apparently cured. 5. Positive test for hepatitis B virus surface antigen (HBV s Ag) or hepatitis C virus ribonucleic acid (HCV antibody) indicating acute or chronic infection. 6. Condition requiring systemic treatment with either corticosteroids or other immunosuppressive medications within 14 days of study drug administration.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Drug Name: Nivolumab **Trade Names:**

Opdivo, Opdualag **ATC Code(s):** L01FF01 **RxNorm Code:** "" (N/A)

Dosage/Frequency: Nivolumab 240 mg or matching placebo every 2 weeks. **Route of Administration:** Intravenous (IV) infusion.

Duration of Treatment: Maximum treatment duration of 1 year, or until disease recurrence or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications. **Prohibited:** Concurrent systemic immunosuppressive medications, corticosteroids (except standard physiological replacement doses), or other active adjuvant systemic anticancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Imaging for disease-free confirmation, Biomarker tissue sampling, Baseline Labs
Baseline	Day 0	Randomization, First IV Infusion
Interim	Every 2 Weeks	Study drug infusion, safety assessments, monitoring for immune-mediated AEs, standard imaging
End of Study	1 Year / Recurrence	Final safety labs, treatment discontinuation, shift to long-term survival follow-up phase

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes

- 1. Disease Free Survival (DFS):** The time between the date of randomization and the date of first documented recurrence (local urothelial tract, local non-urothelial tract or distant), or death due to any cause, whichever occurs first.
- 2. Disease Free Survival (DFS) in PD-L1 Expression $\geq 1\%$ Population:** The time between the date of randomization and the date of first documented recurrence (local urothelial tract, local non-urothelial tract or distant), or death due to any cause, whichever occurs first.

7.2 Secondary Outcomes

- 1. Non-Urothelial Tract Recurrence Free Survival (NUTRFS)**
- 2. Non-Urothelial Tract Recurrence Free Survival in PD-L1 Expression $\geq 1\%$ Population**
- 3. Overall Survival (OS)**
- 4. Distant metastasis-free survival (DMFS)**
(Exploratory/Tertiary)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard monitoring via investigator assessment. The most common monitored AEs include rash, fatigue, diarrhea, pruritus, musculoskeletal pain, and urinary tract infections. **Serious Adverse Events (SAEs):** Routine SAE reporting timelines (typically within 24 hours). Grade ≥ 3 AEs and immune-mediated toxicities (e.g., pneumonitis) are strictly tracked. **Data Monitoring Committee (DMC):** External independent committee to evaluate interim safety and efficacy data.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes; subgroup analysis for PD-L1 $\geq 1\%$. **Safety Set** for evaluating adverse events. **Sample Size Justification:** Powered to detect a statistically significant improvement in DFS via prespecified alpha distribution between

the ITT population and PD-L1 positive populations. **Handling of Missing Data:** Standard Kaplan-Meier methodologies; patients without documented recurrence or death were censored at the date of their last evaluable disease assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Comprehensive onsite and remote monitoring by the sponsor to ensure protocol adherence and patient safety.

Audit Trail: Validated Electronic Data Capture (EDC) system including eCRF locking and query resolution forms.

11. Study Identifiers & Governance

Primary ID: CA209-274 **Registry Number:** NCT02632409 **Posting ID:** 265719595791392171 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** CheckMate-274 **EMA URLs:** * https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf * https://www.ema.europa.eu/en/documents/product-information/opdualag-epar-product-information_en.pdf **Source Level:** 5

12. Clinical Framework (Disease Specific)

Disease Areas: Various Advanced Cancer **Primary Condition:** Urothelial Carcinoma (Muscle-invasive, Bladder or Upper Urinary Tract) **Stage/Phase Name:** Advanced (Preferred Name: Advanced phase) **Definition:** Far along in course. **Semantic Type:** Qualitative Concept **SNOMED ID:** 86005002 **SNOMED Hierarchy:** 106234000|General adjectival modifier|General adjectival modifier (qualifier value) **MeSH Codes:** N/A (mesh_codes: []) **HPO Codes:** [] **Diagnosis Threshold:** High risk of recurrence post-radical surgical resection **Medical Methodology:** Adjuvant Immunotherapy **Optimization Target:** Disease-Free Survival (DFS) extension and prolonged Overall Survival (OS)

13. Study Procedures & Methodology

Management Pathway: Standard oncology postoperative surveillance pathway combined with adjuvant immunotherapy **Education Protocol (HCP):** Identification and management of immune-related adverse events (irAEs) **Education Protocol (Patient):** Treatment compliance, symptom reporting (e.g., pneumonitis, colitis symptoms) **Quality Audit Mechanism:** Independent central review of imaging and routine site audits

14. Observation & Follow-Up Schedule

Total Duration: 1 year of treatment; long-term survival follow-up up to 5+ years **Onsite Visit Cadence:** Every 2 weeks during the active treatment phase **Remote Monitoring Frequency:** Standard safety check-ins **Checklist Review Interval:** Routine radiographic tumor imaging reviews (approx. every 12 weeks)

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Insert PI Name]	_____	06-Apr-2026					
Clinical Operations Lead	[Insert COL Name]	_____	06-Apr-2026					
Lead Statistician	[Insert LS Name]	_____	06-Apr-2026					